INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Weekly cetuximab + head and neck radiotherapy

Understand your treatment. Know what to expect. Keep your plan to hand.

Displayed calendar
7 days
See the active treatment phase

Treatment stages
3
Explained in the treatment reference

Regimen source
NCCP 00207
Version 6

First dose 1 week before radiation; then weekly until radiation ends.

Treatment stages

01 **Cetuximab before radiotherapy**
One week before radiation

02 **Weekly cetuximab with radiotherapy**
Weekly until radiation ends

03 **Recovery and mineral checks**
After the last dose

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

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My name _____

Hospital / oncology team _____

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Tell staff immediately about an infusion reaction. After leaving, seek urgent help for swelling, wheeze, faintness or breathing difficulty. Report spreading rash, infected nails, eye pain, visual change, severe diarrhoea or inability to drink. Mouth and skin effects from radiation may continue after its final session.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

How each part of treatment works

Cetuximab is combined with radiotherapy for selected locally advanced head and neck cancer when platinum chemotherapy is unsuitable. The team discusses the reason for choosing this treatment and the alternatives.

EGFR-TARGETED ANTIBODY

Cetuximab

Erbitux



Blocks a growth signal at the cell surface

Cetuximab binds to EGFR, a receptor involved in growth and repair. It helps radiotherapy work against selected head and neck cancers. Healthy skin also uses EGFR, which helps explain acne-like rash, dryness and nail changes.

EGFR receptor blocked → Growth messages reduced → Radiotherapy combination

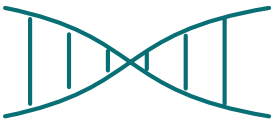
What to watch for

Tell the nurse immediately about itching, flushing, swelling or breathing difficulty. Report rash, painful nails, eye pain or new breathlessness.

LOCAL TREATMENT

Radiotherapy

Radiotherapy



Damages cancer cells in the planned treatment area

Radiotherapy directs radiation at a carefully planned area. It is delivered on its own appointment calendar. Effects depend on which tissues receive radiation and can build up during the course or continue after it finishes.

Treatment area planned → Repeated local treatment → Recovery and late-effect review

What to watch for

Report difficulty swallowing, weight loss, mouth pain, skin breakdown or problems managing saliva.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Understanding your diagnosis

Keep your pathology and agreed treatment goal with your records.

Site, type and stage

Ask which part of the head or neck the cancer started in, what the biopsy shows and how far it has spread. Site, stage, previous treatment and your health affect the options. HPV/p16 testing is relevant to some sites; it is not the same test as PD-L1.

Which pathway applies?

Locally advanced squamous head and neck cancer in people ineligible for platinum-based treatment. This guide should match the named treatment on your prescription.

What tumour tests mean

Your team explains any HPV/p16, Epstein–Barr virus or other tumour tests relevant to your cancer site. Not every head and neck cancer needs the same tests; the medicine safety checks are a separate matter.

Understanding the evidence

The Bonner trial compared radiotherapy with or without cetuximab in locally advanced squamous head and neck cancer.

The study supports adding cetuximab to radiation in an appropriate setting. It did not compare cetuximab-radiotherapy with cisplatin-radiotherapy and does not establish that the treatments are equivalent.

CHAPTER 02

Your schedule, tests and appointments

IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

Treatment stages

Treatment stages

01 **Cetuximab before radiotherapy**
One week before radiation

02 **Weekly cetuximab with radiotherapy**
Weekly until radiation ends

03 **Recovery and mineral checks**
After the last dose

Your team confirms the active stage and dates. The full treatment guide explains each step.

This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

ONE WEEK BEFORE RADIATION

Cetuximab before radiotherapy

First dose

Cetuximab 400 mg/m² over 120 minutes, with prescribed premedication, monitoring and observation.

WEEKLY UNTIL RADIATION ENDS

Weekly cetuximab with radiotherapy

Weekly visits

Cetuximab 250 mg/m² over 60 minutes every week while radiotherapy continues. The team coordinates the final dose with the radiation plan.

AFTER THE LAST DOSE

Recovery and mineral checks

Month 1 **Month 2**

NCCP specifies monthly electrolytes, magnesium and calcium for two months after the final cetuximab dose; continue mouth, skin, swallowing and nutrition care as needed.

Dates and treatment decisions come from your team. A delayed or omitted dose does not count as treatment received.

BASELINE → TREATMENT → FOLLOW-UP

My treatment, tests and checks

Source-required blood tests are distinguished from tests ordered for a particular symptom or disease plan. Imaging is not interchangeable with blood safety tests.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00207 v6: official regimen

Tests in your plan

Full blood count, kidneys and liver

WHEN

Baseline and regularly during treatment

Support safe treatment and identify problems needing review.

Electrolytes, magnesium and calcium

WHEN

Monitor during treatment; monthly for two months after the last dose.

Checks a medicine-specific risk that symptoms alone may miss.

THE WHOLE TREATMENT JOURNEY

Weekly cetuximab + head and neck radiotherapy: treatment and checks

Cetuximab starts one week before radiotherapy: a 400 mg/m² first infusion, then 250 mg/m² every week during radiation until radiotherapy ends. The number of weekly doses follows the actual radiotherapy plan.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	Treatment visit	Between visits	Phase / dose review	After treatment
Blood and organ tests NCCP	Blood count, kidney and liver function with electrolytes, magnesium and calcium. Review allergies, previous reactions, red-meat allergy or tick bites. Before head and neck radiation, arrange dental, nutrition and swallowing assessment with the team.	Blood count, kidney/liver function and minerals are reviewed regularly as specified in the treatment plan. Vitals and symptoms are checked around infusions; skin, eyes, breathing and hydration are reviewed. Radiation has its own weekly support and assessment plan.	Contact the team if symptoms develop; extra tests may be needed before the next visit.	Recheck relevant results after a hold or changed plan; previous toxicity remains relevant.	Electrolytes, magnesium and calcium monthly for two months after the final cetuximab dose. Continue skin, dental, nutrition, swallowing and radiation follow-up. Scans and examinations follow the individual cancer plan; no universal scan date or tumour-marker test is required by this regimen.
Radiotherapy NCCP	Planning appointment and separate radiation calendar.	Cetuximab begins one week before radiation, then continues weekly until radiation ends.	Attend radiation on its own calendar; no assumption of weekend sessions.	Coordinate interruptions and recovery between both teams.	Late effects and response follow-up belong to the radiation plan.

Care / check	Before starting	Treatment visit	Between visits	Phase / dose review	After treatment
Treatment and appointments NCCP	Confirm the exact diagnosis, intent, prescription and calendar.	The first cetuximab infusion runs over 120 minutes; later infusions usually run over 60 minutes. Premedicatio n and observation add to visit time. Your team confirms the observation period; no early discharge is promised after two doses.	Use the prescribed support plan and contact route.	NCCP specifies monthly electrolytes, magnesium and calcium for two months after the final cetuximab dose; continue mouth, skin, swallowing and nutrition care as needed.	Record the final dose and follow-up appointments.
Eating, mouth and swallowing NCCP	Agree oral care and assess weight, intake and swallowing; arrange dental review before head/ neck radiation.	Review pain, weight, hydration and safe swallowing.	Report poor intake, choking, weight loss or inability to drink promptly.	Dietitian and speech/ language plans adapt to symptoms and recovery.	Continue dental, nutritional and swallowing follow-up as needed.
 **Imaging and tumour assessment** NCCP	Confirm staging and any relevant tumour tests; these are distinct from blood safety tests.	Discuss symptoms and treatment response.	The team arranges scans or examinations when indicated.	Use the agreed assessment to guide the next phase.	Record scan and clinic dates; this regimen does not impose a single timetable for everyone.

What each check is for

1 **Cetuximab before radiotherapy**

One week before radiation

Cetuximab is combined with radiotherapy for selected locally advanced head and neck cancer when platinum chemotherapy is unsuitable. The team discusses the reason for choosing this treatment and the alternatives.

Ask / **act:** Confirm the current phase, prescription, tests and support plan.

2 **Weekly cetuximab with radiotherapy**

Weekly until radiation ends

Cetuximab is combined with radiotherapy for selected locally advanced head and neck cancer when platinum chemotherapy is unsuitable. The team discusses the reason for choosing this treatment and the alternatives.

Ask / **act:** Confirm the current phase, prescription, tests and support plan.

3 **Recovery and mineral checks**

After the last dose

Cetuximab is combined with radiotherapy for selected locally advanced head and neck cancer when platinum chemotherapy is unsuitable. The team discusses the reason for choosing this treatment and the alternatives.

Ask / **act:** Confirm the current phase, prescription, tests and support plan.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Practical treatment planning

Agree the arrangements with your oncology team.

STEP 1

Before the first visit

Confirm the exact regimen, treatment aim, baseline tests, transport, support medicines and actual urgent contact numbers. Bring any hearing concerns, swallowing problems and medicine list.

STEP 2

Time in the unit

The first cetuximab infusion runs over 120 minutes; later infusions usually run over 60 minutes. Premedication and observation add to visit time. Your team confirms the observation period; no early discharge is promised after two doses.

STEP 3

Two departments, one calendar

Radiation appointments are separate from medicine visits. Keep one calendar showing both departments and tests; tell the team promptly about a cancelled session.

STEP 4

Practical help

Tell the team about travel, cost, work, caring responsibilities, communication or reading needs. Ask whether a support person or interpreter can attend and how treatment changes will be explained.

STEP 5

Leaving the unit

Check the next visit and blood-test dates, support medicines and who to call if unwell. Have the team replace example phone numbers before using the wallet card.

Recovery, assessment and longer-term care

Treatment completion and recovery are different milestones.

Continuing symptoms

Electrolytes, magnesium and calcium monthly for two months after the final cetuximab dose. Continue skin, dental, nutrition, swallowing and radiation follow-up. Scans and examinations follow the individual cancer plan; no universal scan date or tumour-marker test is required by this regimen.

Cancer assessment

Your team sets examinations and imaging around the disease, treatment and symptoms. Record who arranges each test and who explains the result. An unbooked scan is not automatically due at a fixed week because a general guide says so.

Information for other clinicians

Keep the regimen name, last treatment date, ongoing medicines and relevant alert card. Tell clinicians about earlier immune treatment or head and neck radiation even after the course has ended.

CHAPTER 03

Symptoms and supportive care

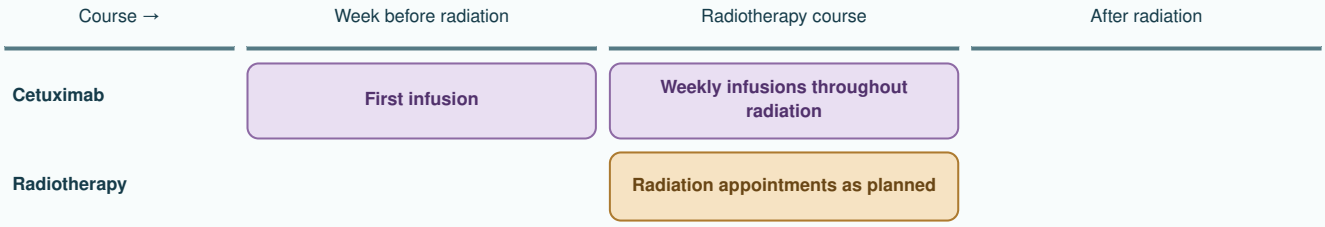
IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

THE WHOLE TREATMENT COURSE

Head-and-neck indication: cetuximab before and during radiotherapy

This panel applies only to the head-and-neck radiotherapy indication of NCCP 00207. Cetuximab is a targeted antibody. Colorectal cetuximab alone has no routine radiotherapy component.



The axis shows order, not equal durations. Cetuximab starts one week before radiation and continues until radiation ends. The radiation team sets the number of sessions and confirms eligibility.

> **After the course: continue skin, mouth, swallowing and nutrition support. Mineral monitoring continues for two months after the final cetuximab dose.**

Educational reference, not a personal appointment calendar. NCCP 00207 v5, page 1; current EMA Erbitux information

Day 1 → Day 7: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Treatment or check	Timing in this view
Cetuximab infusion	 **Day 1** Weekly reference visit; the initial loading dose is one week before radiation.
Radiotherapy appointments	 **Days 1–7** Concurrent phase only. This band shows overlap, not seven radiation sessions.
Skin, mouth and nutrition care	 **Days 1–7** Continue each day and during recovery.

Treatment sequence: Cetuximab starts one week before radiotherapy: a 400 mg/m² first infusion, then 250 mg/m² every week during radiation until radiotherapy ends. The number of weekly doses follows the actual radiotherapy plan.

Baseline and ongoing checks: Blood count, kidney and liver function with electrolytes, magnesium and calcium. Review allergies, previous reactions, red-meat allergy or tick bites. Before head and neck radiation, arrange dental, nutrition and swallowing assessment with the team. Blood count, kidney/liver function and minerals are reviewed regularly as specified in the treatment plan. Vitals and symptoms are checked around infusions; skin, eyes, breathing and hydration are reviewed. Radiation has its own weekly support and assessment plan.

Appointments and administration: The first cetuximab infusion runs over 120 minutes; later infusions usually run over 60 minutes. Premedication and observation add to visit time. Your team confirms the observation period; no early discharge is promised after two doses.

Recovery and follow-up: Electrolytes, magnesium and calcium monthly for two months after the final cetuximab dose. Continue skin, dental, nutrition, swallowing and radiation follow-up. Scans and examinations follow the individual cancer plan; no universal scan date or tumour-marker test is required by this regimen.

> **Why these bands stay broad:** The days show a usual treatment pattern, not a prediction of when symptoms will happen. Your team confirms dates and any changes. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00207 v6: official regimen ↗

> **There are no guaranteed "safe days".** Your prescription and team's instructions take priority. Contact the team about new symptoms rather than waiting for a scheduled visit.

Between-treatment notes

Day 1: Cetuximab visit

The first cetuximab infusion runs over 120 minutes; later infusions usually run over 60 minutes. Premedication and observation add to visit time. Your team confirms the observation period; no early discharge is promised after two doses. Report new rash, eye, bowel or breathing symptoms before treatment.

Day 2–7: Care between weekly visits

Follow the radiation calendar and the mouth, skin and nutrition plan. Report problems promptly; symptoms may build over successive weeks.

Support around the medicines

1 

One written medicine plan

Ask for one schedule showing prevention medicines, rescue treatment and what to do if symptoms persist. Do not add, double or restart medicines from an old prescription.

2

Premedication, skin and minerals

Premedication helps reduce infusion reactions; the team also provides a skin-care plan. Keep mineral blood-test appointments, including monthly checks for two months after the final dose. Report delayed reactions even after earlier infusions were tolerated.

3 Food, fluids and weight

A dietitian can tailor food texture, supplements and calorie or protein needs. Tell the team early if eating or drinking becomes difficult; do not wait for substantial weight loss. Fluid targets depend on kidney and heart health.

4 Oral and dental care

Use the mouth-care plan provided by the team and tell them about ulcers, thrush, bleeding or pain. Inform your dentist about cancer treatment. Before any dental procedure, confirm timing with the oncology team.

5 Swallowing and communication

A speech and language therapist can assess swallowing and advise exercises or safer food and drink textures. Follow the individual assessment; a general guide cannot decide which textures are safe for you.

6 Fertility, pregnancy and medicines

Discuss fertility and pregnancy precautions before treatment and the medicine-specific duration of contraception afterwards. Ask about breastfeeding. Bring a full medicine, supplement and allergy list; check before adding over-the-counter or herbal products.

Treatment choices and your priorities

The consent conversation explains the boundaries of the plan.

Benefit and alternatives

Cetuximab is combined with radiotherapy for selected locally advanced head and neck cancer when platinum chemotherapy is unsuitable. The team discusses the reason for choosing this treatment and the alternatives. Ask how alternatives compare for your health, goals and previous treatment. A study result cannot predict an individual outcome.

A pause is a treatment decision

The clinician may delay, change or stop treatment because of symptoms, test results or your priorities. Record which component is affected and what is needed before restarting. Do not restart from an old prescription.

Combining treatment approaches

Radiotherapy treats a planned local area while medicine supports the cancer treatment. The radiation prescription, number of sessions and late-effect plan are agreed separately.

Preparing for and receiving treatment

Plan the appointments and support around the prescribed schedule.

If an appointment changes

Contact the unit for revised dates after a missed or delayed visit. A missed appointment is not a completed dose. Ask whether blood tests, premedicines or pump appointments also need to change.

What the record shows

Write the actual treatment date, phase and any change. The wallet card is a brief record; the passport holds the fuller history. It does not replace the hospital treatment record.

Between visits: symptoms and medicines

Use the agreed home plan and contact pathway.

Symptoms have no fixed calendar

Some effects appear soon after treatment and others build over weeks. Do not wait for the next appointment if a symptom is new, severe or worsening.

A medicine plan you can use

Keep prevention, rescue and regular medicines clearly separated on one written list. Record what you have taken and the response, so the team can adjust it. Do not double a missed dose unless your prescriber tells you to.

Mouth, teeth and jaw care

Dental preparation and gentle daily care reduce avoidable problems.

Before treatment

Arrange dental assessment before head and neck radiation. The dental and oncology teams decide what treatment is needed and how much healing time is required; do not schedule extractions independently.

During treatment

Follow the prescribed brushing, fluoride, rinse and pain-relief advice. Report ulcers, infection, new bleeding, difficulty wearing dentures or worsening jaw opening. Avoid starting a new mouthwash without checking if the mouth is sore.

After head and neck radiation

Dry mouth can increase dental problems and some radiation effects last. Keep regular dental care and tell any future dentist that you have had head and neck radiation before invasive treatment. Your radiation/dental team plans any procedure.

Eating, drinking and maintaining strength

The aim is adequate intake and comfort during treatment.

Recognising an intake problem

Record weight and report food or drink becoming harder to manage. Pain, taste change, dry mouth or nausea can each need a different response; a dietitian can adapt the plan to you.

If ordinary food is not enough

The team may suggest food fortification, supplements or tube feeding when intake or swallowing safety requires it. A tube is a support option, not a judgement about how well you have tried to eat. Training and follow-up are needed.

Fluid needs are individual

Ask for a realistic drinking plan that accounts for kidney/heart problems and any feeding tube. Persistent vomiting, dizziness, markedly reduced urine or inability to drink needs urgent contact.

Swallowing, voice and communication

A speech and language therapist can help protect function.

Swallowing assessment

Report coughing or choking with food or drink, a wet-sounding voice after swallowing, food sticking or repeated chest infections. The team may need to assess aspiration risk and safe textures.

Exercises and rehabilitation

Do swallowing or jaw exercises only as taught for your situation. Treatment and recovery can change what is comfortable and safe. Ask for reassessment when symptoms change.

Communication needs

Tell the team about changes in speech, voice or hearing. Ask for written instructions, communication aids, an interpreter or help involving a support person.

Daily life and emotional support

Practical adjustments can make the plan easier to follow.

Energy and activity

Balance rest with manageable activity according to your ability and team advice. Ask for help if weakness, falls, sleep difficulty or pain interferes with daily life.

Smoking and alcohol

Ask for support to stop smoking and discuss alcohol with the team. Mouth irritation, medicine interactions and head and neck recovery can change what is safe for you.

Emotional and practical concerns

Tell the team about anxiety, appearance or intimacy concerns, finances, travel and caring responsibilities. Ask which local supports are available; resources can be added to your record when confirmed.

Your records and additional help

Keep essential instructions available on paper.

The passport and wallet card

The three-page companion gives the essentials. Use the passport for the agreed plan, visits, test results, symptoms and questions. The wallet QR opens this exact treatment's further information; it contains no personal clinical record.

Contacts before patient use

The phone numbers printed as examples must be replaced and checked by the treating service. Add the unit's daytime, out-of-hours and backup route; keep this information where you and a support person can find it.

If you prefer paper

Use the full printable handbook in its chapter order and write questions in the passport. You do not need to scan the QR or follow an internet link to use the essential treatment and safety information.

Questions worth asking

Which tests are required?

Blood count, kidney and liver function with electrolytes, magnesium and calcium. Review allergies, previous reactions, red-meat allergy or tick bites. Before head and neck radiation, arrange dental, nutrition and swallowing assessment with the team. Blood count, kidney/liver function and minerals are reviewed regularly as specified in the treatment plan. Vitals and symptoms are checked around infusions; skin, eyes, breathing and hydration are reviewed. Radiation has its own weekly support and assessment plan.

What if a treatment is held?

The team records the reason and recovery or tests needed before restarting. Do not count an omitted dose as completed or restart from an old plan.

> **Try explaining the plan back:** Explain which treatment phase you are in, what happens next, which tests are due and which symptoms would make you call today. Ask the team to clarify anything you cannot explain.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Cetuximab before radiotherapy

Active medicines: Cetuximab

Emetogenic risk: minimal

Medicine	Dose / route	When
Antihistamine and corticosteroid premedication	Clinician/pharmacy to complete	Before the first cetuximab infusion; strongly recommended for subsequent infusions Complete the local authorised prescription and observation plan.

Additional phase-specific support

- Ask for one schedule showing prevention medicines, rescue treatment and what to do if symptoms persist. Do not add, double or restart medicines from an old prescription.
- Premedication helps reduce infusion reactions; the team also provides a skin-care plan. Keep mineral blood-test appointments, including monthly checks for two months after the final dose. Report delayed reactions even after earlier infusions were tolerated.
- A dietitian can tailor food texture, supplements and calorie or protein needs. Tell the team early if eating or drinking becomes difficult; do not wait for substantial weight loss. Fluid targets depend on kidney and heart health.
- Use the mouth-care plan provided by the team and tell them about ulcers, thrush, bleeding or pain. Inform your dentist about cancer treatment. Before any dental procedure, confirm timing with the oncology team.
- A speech and language therapist can assess swallowing and advise exercises or safer food and drink textures. Follow the individual assessment; a general guide cannot decide which textures are safe for you.
- Discuss fertility and pregnancy precautions before treatment and the medicine-specific duration of contraception afterwards. Ask about breastfeeding. Bring a full medicine, supplement and allergy list; check before adding over-the-counter or herbal products.
- NCCP v6 permits ending post-infusion observation after two tolerated doses; EMA Erbitux specifies at least one hour after every infusion. The clinician/pharmacy team must reconcile this difference locally; no automated early-discharge instruction.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

Weekly cetuximab with radiotherapy

Active medicines: Cetuximab

Emetogenic risk: minimal

Medicine	Dose / route	When
Antihistamine and corticosteroid premedication	Clinician/pharmacy to complete	Before the first cetuximab infusion; strongly recommended for subsequent infusions Complete the local authorised prescription and observation plan.

Additional phase-specific support

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Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

What to watch for

You do not need to know which medicine caused a symptom before calling.

1 Eyes

Cetuximab

Report eye pain, redness, light sensitivity or visual change promptly.

2 Mouth / swallowing

Head and neck radiotherapy

Mouth sores, dry mouth, thick saliva or swallowing pain can affect drinking and nutrition; ask for early help.

3 Infusion reaction

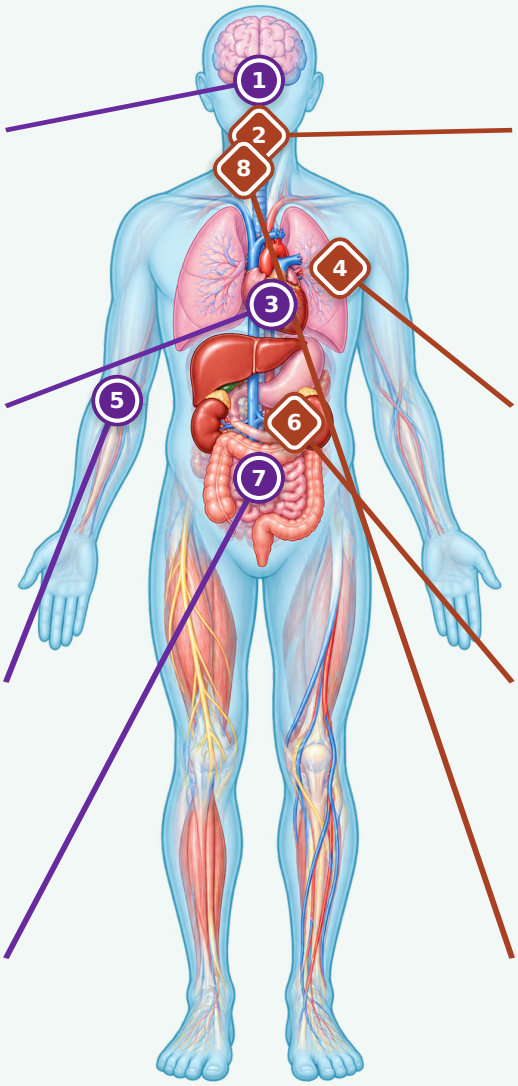
Cetuximab

Tell staff immediately about flushing, itching, swelling, wheeze or faintness; reactions can also occur after leaving.

4 Lungs / breathing

Cetuximab

New cough or breathlessness needs prompt assessment. Severe breathing difficulty is an emergency.



5 Skin / nails

Cetuximab + radiotherapy

Acne-like rash, cracked skin, painful nails or radiation skin breakdown need the agreed care plan and review.

6 Minerals / fluids

Cetuximab

Low magnesium and other minerals may show on blood tests. Report cramps, weakness, palpitations or dehydration.

7 Bowels

Cetuximab

Report diarrhoea, especially with dizziness, much less urine or difficulty drinking.

8 Teeth / jaw / nutrition

Head and neck radiotherapy

Continue dental and swallowing care. Report jaw stiffness, weight loss, choking or a mouth wound that does not heal.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Tell staff immediately about an infusion reaction. After leaving, seek urgent help for swelling, wheeze, faintness or breathing difficulty. Report spreading rash, infected nails, eye pain, visual change, severe diarrhoea or inability to drink. Mouth and skin effects from radiation may continue after its final session.

When to get help

Call 112 / 999 for severe breathing difficulty, severe chest pain, collapse or a seizure.

Call oncology immediately: temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

CHAPTER 04

Your record and contact card

IN THIS CHAPTER

Space for your own cycle or review record, questions and notes, followed by your treatment card.

Record each cycle separately. Each medicine has its own symptom table. Mark the worst symptoms since your last review, when they occurred and what they stopped you doing. Overlapping symptoms can be recorded under more than one medicine; you do not have to identify the cause.

These are patient-language severity prompts, not a validated questionnaire or a formal Common Terminology Criteria for Adverse Events (CTCAE) grading tool. Your team assigns the clinical grade. Serious symptoms override the columns: call without waiting to decide a severity level.

Use a new record sheet for each cycle or review. Copy or print another sheet when needed; the number of sheets does not prescribe the length of treatment.

Cycle number: _____ Start date: _____ Next review: _____

Record actual treatment received, dose changes, tests and contact plans with your team. A stamp records a completed treatment; it does not authorise the next dose.

Cetuximab before radiotherapy · copy this sheet for another cycle of the same phase.

Cetuximab · EGFR-targeted antibody

Symptom	None	Mild	Moderate / call	Severe / urgent	Emergency signs
Skin / nail changes	□ None	□ Mild change: report	□ Pain or daily tasks affected: call	□ Spreading infection or blistering: urgent review	□ Severe illness / collapse: emergency help
Eye symptoms	□ None	□ New irritation: report	□ Pain, redness or light sensitivity: call promptly	□ Blurred vision / severe eye pain: urgent review	□ Sudden major vision loss: emergency assessment
Breathing	□ Usual breathing	□ New cough / change: call	□ Worsening breathlessness: urgent advice	□ Breathless at rest: urgent assessment	□ Severe breathing difficulty: 112/999
Diarrhoea	□ Usual pattern	□ Up to 3 extra stools above normal	□ 4–6 extra stools/ day or at night: call	□ 7+ extra stools/ day or dehydration: urgent review	□ Collapse / severe dehydration: 112/999
Muscle cramps	□ None	□ Mild cramps: record	□ Repeated cramps: contact team	□ Severe pain / weakness or dark urine: urgent review	□ Sudden major weakness / collapse: 112/999
Infusion symptoms	□ None	□ New flushing, chills or itching: tell the nurse immediately	□ Symptoms during infusion: tell the nurse immediately	□ Wheeze, swelling or faintness: immediate urgent help	□ Severe breathing difficulty or collapse: emergency help
Fatigue	□ Usual energy	□ Tiredness improves with rest	□ Rest does not help; usual tasks affected: call	□ Self-care difficult: urgent review	□ Collapse: 112/999

Dates / doses missed or changed / symptoms affecting daily life / questions:

Red flags override the table. Fever, shivers, suddenly unwell, chest symptoms or inability to drink: seek prompt advice. Severe breathing difficulty, collapse or severe chest pain: 112 / 999.

Cycle number: _____ Start date: _____ Next review: _____

Record actual treatment received, dose changes, tests and contact plans with your team. A stamp records a completed treatment; it does not authorise the next dose.

Weekly cetuximab with radiotherapy · copy this sheet for another cycle of the same phase.

Cetuximab · EGFR-targeted antibody

Symptom	None	Mild	Moderate / call	Severe / urgent	Emergency signs
Skin / nail changes	□ None	□ Mild change: report	□ Pain or daily tasks affected: call	□ Spreading infection or blistering: urgent review	□ Severe illness / collapse: emergency help
Eye symptoms	□ None	□ New irritation: report	□ Pain, redness or light sensitivity: call promptly	□ Blurred vision / severe eye pain: urgent review	□ Sudden major vision loss: emergency assessment
Breathing	□ Usual breathing	□ New cough / change: call	□ Worsening breathlessness: urgent advice	□ Breathless at rest: urgent assessment	□ Severe breathing difficulty: 112/999
Diarrhoea	□ Usual pattern	□ Up to 3 extra stools above normal	□ 4–6 extra stools/ day or at night: call	□ 7+ extra stools/ day or dehydration: urgent review	□ Collapse / severe dehydration: 112/999
Muscle cramps	□ None	□ Mild cramps: record	□ Repeated cramps: contact team	□ Severe pain / weakness or dark urine: urgent review	□ Sudden major weakness / collapse: 112/999
Infusion symptoms	□ None	□ New flushing, chills or itching: tell the nurse immediately	□ Symptoms during infusion: tell the nurse immediately	□ Wheeze, swelling or faintness: immediate urgent help	□ Severe breathing difficulty or collapse: emergency help
Fatigue	□ Usual energy	□ Tiredness improves with rest	□ Rest does not help; usual tasks affected: call	□ Self-care difficult: urgent review	□ Collapse: 112/999

Radiotherapy · Local treatment

Symptom	None	Mild	Moderate / call	Severe / urgent	Emergency signs
Mouth soreness / ulcers	□ None	□ Mild soreness; eating normally	□ Painful; soft food needed: call	□ Cannot eat / drink enough: urgent review	□ Major breathing difficulty: 112/999
Taste / appetite	□ Usual eating	□ Taste change: record	□ Eating less / weight falling: contact team	□ Unable to eat or drink enough: urgent review	□ Collapse / severe dehydration: 112/999
Weight	□ Usual weight	□ Small change: record	□ Ongoing loss / swelling: contact team	□ Rapid change with breathlessness: urgent review	□ Severe breathing difficulty / collapse: 112/999
Skin	□ No change	□ New small change: report	□ Persistent rash / itch: contact team	□ Painful, spreading or blistering rash: urgent review	□ Severe skin reaction with acute illness: emergency help
Fatigue	□ Usual energy	□ Tiredness improves with rest	□ Rest does not help; usual tasks affected: call	□ Self-care difficult: urgent review	□ Collapse: 112/999

Dates / doses missed or changed / symptoms affecting daily life / questions:

Red flags override the table. Fever, shivers, suddenly unwell, chest symptoms or inability to drink: seek prompt advice. Severe breathing difficulty, collapse or severe chest pain: 112 / 999.

Keep your treatment details to hand

Locally advanced squamous head and neck cancer in people ineligible for platinum-based treatment. · NCCP 00207 v6 · Document revised 10 November 2026

Print this page **one-sided on A4 at Actual size / 100%**. Cut only the dashed outer border around both panels. **Do not cut the centre line.** Fold with FRONT and BACK facing out, then glue or tape the blank backs together. Finished size: **85.6 × 54 mm**. Keep the stamp side writable.

FRONT / CONTACTS

My Treatment Card

SACTCheck

Name: _____

Cetuximab + RT

First dose 1 week before radiation; then weekly until radiation ends.

Phase: _____ Cycle plan: team to confirm

Daytime (EXAMPLE) 087 12345678
Out of hours (EXAMPLE) 087 87654321

Unwell or fever? Call oncology now.
Severe breathing difficulty, chest pain or collapse: **112 / 999.**

EXAMPLE NUMBERS — replace before patient use.

BACK / TREATMENT RECORD

My cycle record

Rolling weekly record through the radiation course.

Cycle	Date / initials

My treatment information

Latest treatment date: _____

Plan changed? Ask the team to update this card.

CUT outer border

FOLD centre — do not cut

Keep the record accurate

The team dates and initials each recorded treatment unit when complete. A cycle is complete after its final planned dose; a weekly-treatment stamp records that weekly dose. Delays, omitted doses or a changed plan need a note or a replacement card. A stamp does not approve the next treatment. Keep any medicine-specific alert card too.

Put your treatment page on your phone

Scan your QR and check the treatment name. Save that page to your home screen.

iPhone / Safari

Tap Share → Add to Home Screen. Select Open as Web App if offered, then Add.

Android / Chrome

Tap the three-dot menu → Add to Home screen or Install and create shortcut. Choose the shortcut option and confirm.

Menu wording varies. The QR opens information, not a personal record. Paper stamps do not update the website. Check that the saved icon opens the correct treatment.

Your local contacts and resources

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

Hospital / team: _____
Backup if the team cannot be reached: _____

Additional local services and resources will be added when confirmed. Your team must check and complete this page before patient use.

SACTCheck v0.84.47 · NCCP 00207 v6 · Wallet-card sheet
Feasibility build. Clinical, pharmacy and patient/PPI review pending. Your team confirms your personal plan.

CHAPTER 05

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **NCCP 00207 v5, page 1; current EMA Erbitux information**
   https://assets.hse.ie/media/documents/207_v5_Cetuximab_7day.pdf
2. **NCCP 00207 v6 · pSee regimen treatment, tests and supportive-care sections**
   https://healthservice.hse.ie/documents/6941/207_Cetuximab_7day_.pdf
3. **NCCP head and neck regimen catalogue**
   https://healthservice.hse.ie/staff/information-healthcare-workers/nccp/head-and-neck-sact-regimens/
4. **NCI: head and neck cancers**
   https://www.cancer.gov/types/head-and-neck/head-neck-fact-sheet
5. **NCI: oral complications of cancer therapies, patient version**
   https://www.cancer.gov/about-cancer/treatment/side-effects/mouth-throat/oral-complications-pdq
6. **NCI: nutrition during cancer treatment**
   https://www.cancer.gov/about-cancer/treatment/side-effects/nutrition
7. **HSE: cancer treatment side effects**
   https://www.hse.ie/conditions/cancer/if-you-have-side-effects/
8. **Irish Cancer Society: head and neck radiotherapy effects**
   https://www.cancer.ie/cancer-treatments/radiotherapy/side-effects-from-radiotherapy-to-the-head-and-neck-area
9. **EMA: Erbitux product information and patient leaflet**
   https://www.ema.europa.eu/en/documents/product-information/erbitux-epar-product-information_en.pdf
10. **Bonner et al: radiotherapy with or without cetuximab, 2006**
    https://pubmed.ncbi.nlm.nih.gov/16467544/
11. **NCCP antiemetics v8, Table 1 pp4–5**
    https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf